



KRIYA AI

CEO-LEVEL STRATEGIC TRANSFORMATION BLUEPRINT · 2026 - 2040

KRIYA AI 2040

From Healthcare Intelligence Layer to Autonomous Business Infrastructure

A first-principles plan for Kriya AI and Kriya Omnitask. It sets out the current baseline, market reality in India and globally, the category to create, the architecture, six phases to 2040, scenarios, 34 failure modes, and the next 36 months.



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EVIDENCE STANDARD USED THROUGHOUT

FACT a sourced, current fact **FORECAST** a forecast attributed to a named source **HYPOTHESIS** my strategic reasoning, to be tested **KRIYA SCENARIO** a planned state for Kriya, not a prediction. Sources are numbered [S1]–[S24] (Section 36). Where evidence is missing, the text says “**Evidence insufficient.**” Revenue ranges, scores and probabilities are hypotheses unless marked otherwise.

DECISION RECORDED FOR THIS BLUEPRINT

Xylarc Omnitask becomes Kriya Omnitask. It moves out from under the Xylarc AI brand and becomes the autonomous-operations engine of the Kriya platform. The healthcare product becomes **Kriya Health**, the first solution built on it. The remaining question, whether Xylarc AI stays as the legal entity or the company itself becomes Kriya, is Decision #1 in Section 34.



01

SECTION 01

Executive Summary

Kriya AI today is two things. **Kriya Health** is a carefully engineered WhatsApp operations system for clinics, hospitals and diagnostic labs, now onboarding its first hospital and with a corporate-health proposal before a diagnostic chain. **Kriya Omnitask** is an advanced horizontal agent runtime with orchestration, risk gates, an audit ledger and a model router, but no documented production customers yet. The strengths are real. The market around them moved sharply in 2026.

Jun 2026

Meta's own Business Agent went global: it books appointments, qualifies leads and hands off to humans on WhatsApp [S14]

\$1.5B+

Salesforce Agentforce ARR, priced at roughly \$0.10 per standard agent action [S15]

>40%

of agentic AI projects forecast to be cancelled by end-2027 (Gartner) [S1]

May 2027

DPDP substantive obligations apply to every data fiduciary in India [S5]

THE CENTRAL CONCLUSION

Kriya should not sell "AI agents". It should create and own Verified Action Infrastructure.

By 2030, agents that talk and click will be cheap and supplied by Meta, the model labs and the enterprise suites. What stays scarce is **a consequential action that several organisations will accept**: an admission, a claim, a refund, a disclosure, a payment. That means an action that is authorised, bounded, executed across systems, verified and provable. Kriya's deterministic core, clinical firewall, zero-retention parsing, audit ledger and risk gates are the seed of that. Healthcare, where these actions are frequent, regulated and multi-party, is the right place to prove it. **HYPOTHESIS**

ARCHITECTURE

Omnitask becomes the runtime

Kriya Omnitask runs orchestration and policy. Proof, Mandate and Lens are the trust modules. Kriya Health becomes the first solution pack, not the master identity.

EXPANSION LOGIC

Across the value chain, not across random sectors

Providers → employers → claims and revenue cycle → consent and compliance → pharma → other claims-heavy sectors (warranty, general insurance, lending documents).

ENTERPRISE ENTRY

Dated, budgeted, multi-party problems

Group-health renewals, hospital claim deductions, DPDP rights handling. Land with paid pilots that have success criteria, then expand by department.

WHAT WAS REVISED

Challenged and changed

The SMB booking tier is threatened by Meta. The claims thesis moves from insurer SLA penalties to hospital-side deductions. Browser automation is downgraded as a moat because computer-use agents are commoditising it.

Full CEO conclusion (points A–J) in Section 35. The ten decisions with deadlines are in Section 34.



02 SECTION 02

Current Kriya AI Baseline

Status is taken from your documentation and engineering record. **“Documented” means the docs describe it but I have not seen production evidence.** No feature is treated as production-proven only because a document says it exists.

A · Kriya Health (formerly MediAssist, “Kriya AI”)

CAPABILITY	EVIDENCE	STATUS
WhatsApp conversational OPD booking (English/Hindi/Telugu), state-machine governed	Master doc; engineering history	IMPLEMENTED
Zero-LLM clinical firewall (emergency markers, dosage requests → escalation)	Master doc (clinical_firewall.py)	DOCUMENTED
ACID slot holds, Razorpay links with HMAC-SHA256 verification, refunds	Built module; CLAUDE.md spec	IMPLEMENTED
Doctor-leave auto-cancel with UPI refund and one-tap rebooking	Master doc	PARTIAL verify end-to-end in production
Live waiting-room queue tokens on WhatsApp	Master doc	PARTIAL needs reception check-in integration per site
24 background jobs with PostgreSQL compare-and-swap locks	Master doc	DOCUMENTED load behaviour unverified
Lab report delivery: MocDoc via Playwright; CallMedex LIS sync	Built integration	IMPLEMENTED CallMedex is a sister platform, not third-party validation
AI bilingual, non-diagnostic report summaries	Master doc	DOCUMENTED needs clinical review protocol
Corporate Health Insights: 18 parameters, zero raw PDF and zero PII stored, cohort guard	Session 27, migrations 095/096	IMPLEMENTED first commercial use pending
Tenant isolation audit (clinic_id scoping), 7-role RBAC, idempotency, dead-letter queue, plan feature gating	Engineering hardening record	IMPLEMENTED
FHIR R4 / ABDM interoperability; DPDP PII-sanitisation middleware	Documented as complete in proposals	VERIFY confirm ABDM milestone scope and get external review
Admin console (18 tabs) and Platform Owner console	Master doc	DOCUMENTED
79-test automated suite	Engineering record	IMPLEMENTED measure coverage on payment and firewall paths
Revenue-leakage detection module	Explored as upsell	PLANNED
Legal, Finance, Education, Estates, Civic sector packs	Master doc §5	CONCEPTUAL
Outcome claims: “no-shows cut by over half”, “60–80% front-desk waste”, “~300 reports in 5 minutes”	Master doc	VERIFY no measured customer data yet



B · Kriya Omnitask (formerly Xylarc Omnitask)

CAPABILITY	STATUS
Hierarchical orchestrator with typed (Zod) inter-agent schemas	IMPLEMENTED founder-reported
Specialist agents: lead qualification (BANT), calendar booking, support/RAG, reactivation & retention, lifecycle controller	IMPLEMENTED founder-reported
Agent Safety Firewall; Human Attention Center with 4-tier risk gates	IMPLEMENTED
SHA-256 cryptographic audit ledger	IMPLEMENTED
Omnichannel gateway: WhatsApp Cloud API, Twilio voice (STT/TTS), email, web	IMPLEMENTED
Dynamic model router (OpenRouter, Anthropic, Gemini, OpenAI, Groq, Ollama) with cost tracking	IMPLEMENTED
Business Digital Twin; hybrid RAG knowledge fabric; DAG workflow engine	IMPLEMENTED
React 19 customer console and platform-operator console	IMPLEMENTED
v2: owner-panel-only provisioning, business-type-driven activation, failure-escalation chains, brain-body separation, secured web search, new design system	PLANNED in progress
Bring-your-own model key commercial model with compatibility checks; managed-key option	PLANNED designed
Production customers, uptime record, security audit	VERIFY none documented

C · Commercial baseline

ITEM	CURRENT STATE
Pricing	SoloClinic ₹1,999 · Essential ₹4,999 · DiagStream ₹6,999 (per branch) · Polyclinic ₹9,999 · Enterprise ₹19,999 per month; 2,000 utility messages included, ₹1,000 per extra 2,000
Customers	First hospital client onboarding; Taiyo Labs corporate-health proposal submitted. No public case study with measured outcomes.
Platform status	Meta App Review resubmission for multi-tenant messaging in progress
Team	Solo founder, full-time student through 2027; several parallel products (Kriya Health, Omnitask, CallMedex, GST copilot concept, other projects)
Stacks	Kriya Health: Python FastAPI · Supabase · Groq · Render. Omnitask: TypeScript-typed agents · React 19 · Supabase · Render/Vercel. Two codebases to converge.

BASELINE VERDICT

Engineering depth is well above typical early-stage products. Market proof is close to zero. The gap between the two is the single most important thing to close. It matters more than any new capability. **HYPOTHESIS**



03

SECTION 03

Kriya's Transferable Technology Assets

ASSET	INDUSTRY-INDEPENDENT?	WHERE IT TRANSFERS	DURABILITY AS MODELS IMPROVE	BECOMES
Deterministic state machine; LLM limited to intent	Yes	Any regulated workflow	High. Rules and audit stay necessary as models improve	Omnitask Policy Engine
Domain safety firewall (clinical)	Pattern yes; rules no	Finance, pharma, legal, insurance	High. Regulators require explicit guardrails	Firewall packs per vertical
Slot-hold + payment + refund ledger	Yes	Any time, money or resource commitment	Med-High	Kriya Mandate
Zero-retention document intelligence	Yes	HR, insurance, lending, KYC documents	High. Privacy law trend favours it	Kriya Lens
SHA-256 audit ledger (Omnitask)	Yes	Every consequential action	High. Evidence value grows with regulation	Kriya Proof
Hierarchical orchestrator + 4-tier risk gates + Human Attention Center	Yes	All agent work	Medium. Platforms ship orchestration; the gates and policy differentiate	Omnitask Runtime
Browser automation into legacy HMS (Playwright)	Yes	Any portal without an API	Low-Med. Computer-use agents are improving fast [S17]	Kriya Reach (wraps computer-use models)
Model router, brain-body separation	Yes	All	Low as a moat, but essential for cost and independence	Omnitask Router
Omnichannel gateway (WhatsApp, voice, email, web)	Yes	All	Low. CPaaS commodity; Meta controls WhatsApp	Integrate, don't differentiate
Healthcare domain knowledge (specialty taxonomies, lab formats, HMS quirks, NMC rules)	No	Health, insurance, pharma, employers	High inside the health value chain	Kriya Health pack
Multilingual (English/Hindi/Telugu)	Partly	All India	Low. Sarvam and frontier models cover it [S10]	Partner
Multi-tenant RBAC, feature gating, idempotency	Yes	All	Low. Table stakes	Platform base

WHAT THE TABLE SAYS

The **durable** assets are the ones that decide, authorise, prove and protect: the state machine, firewall, mandate ledger, zero-retention parsing, audit ledger, and domain knowledge. The **non-durable** ones are those that talk and click: channels, languages, models, browser automation. Build on the first group. Rent the second group. **HYPOTHESIS**



04 SECTION 04 The Strategic Problem

How does Kriya become a distinctive, defensible company when conversation, clicking and generic agent orchestration are being commoditised from above and below at the same time?



First-principles answers

1 · What is Kriya fundamentally good at?

Turning messy human requests into **bounded, verifiable transactions** (a slot, a payment, a report, a disclosure) inside a regulated domain.

2–3 · What transfers and what doesn't

These transfer: the decision engine, firewall pattern, commit ledger, zero-retention parsing, orchestration and audit. These are health-specific: clinical taxonomies, lab templates, NMC rules and HMS adapters.

4–5 · The smallest reusable core

One primitive: **the Verified Action**. An intent is understood, checked against policy and authority, executed across systems, verified, receipted, and escalated if uncertain. Every vertical product is a set of these.

6–7 · What survives cheap, brilliant models?

Authority, liability, proof, integrations into messy multi-party systems, domain policy, relationships and outcome data. Models make these more valuable, not less.

8 · What enterprises will pay for in 2030

Outcomes with guarantees: lower cost-to-serve under an SLA, fewer claim deductions, compliance evidence on demand, measurable turnaround. Not "agents".

HYPOTHESIS

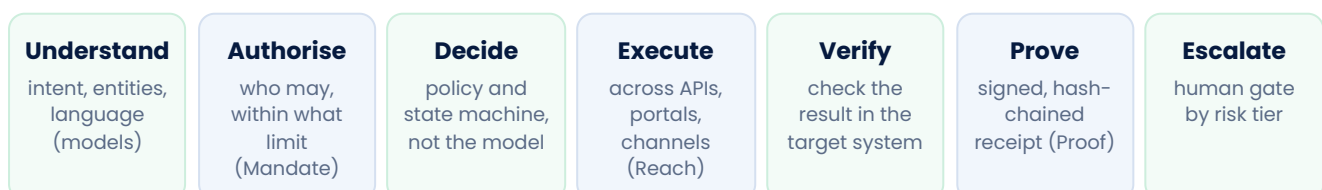
9 · What enterprises will pay for in 2040

Assurance that autonomous actions, their own agents' and their counterparties', are authorised, bounded and provable across institutions. **HYPOTHESIS**

10 · Where trust, execution and compliance beat raw intelligence

Wherever an action moves **money, health information, identity or legal status** between two or more organisations. That describes healthcare, insurance, employee benefits, lending documents, compliance filings and warranty claims. That is Kriya's terrain.

The Verified Action: Kriya's core primitive





05

SECTION 05

The AI / Agent Revolution

A chatbot answers. A copilot assists. An agent executes a task. **An autonomous operational system** perceives, decides, executes, verifies, recovers, escalates, learns and stays auditable. Here is where Kriya actually stands on that ladder:

CAPABILITY	KRIYA HEALTH	KRIYA OMNITASK	IMPLICATION
Perceive (channels, documents)	Strong	Strong	Integrate more channels; don't over-invest
Understand / reason	Adequate (single model)	Strong (router)	Commodity; ride model progress
Plan multi-step work	Weak (fixed flows)	Adequate (DAG, orchestrator)	Needed for claims and compliance
Access systems	Strong in health (HMS, Razorpay)	Adequate	Deepen health and insurance connectors
Execute actions	Strong (bookings, refunds, delivery)	Adequate	Core; keep deterministic
Verify results	Partial	Partial (verification layer)	Key gap: check the target system after acting
Recover from errors	Partial (DLQ, backoff)	Partial (escalation chains)	Build compensating actions
Escalate to humans	Strong (firewall)	Strong (4-tier gates)	Differentiator; keep
Learn from outcomes	Missing	Missing	Key gap: outcome data is the long-term moat
Memory / state	Strong (FSM, ledger)	Strong (digital twin)	Unify in one tenant model
Operate under permissions	Human RBAC only	Human RBAC + risk gates	Key gap: agent identity and delegated authority
Auditability	Partial (logs)	Strong (SHA-256 ledger)	Promote to Kriya Proof
Optimise the workflow over time	Missing	Missing	Phase 2+; needs outcome data first

POSITION ON THE LADDER

Kriya Health is a **reliable task agent** with strong guardrails. Omnitask is an **early multi-agent system**. Neither yet learns from outcomes, verifies results systematically, or carries agent-level authority. Those three gaps define the Phase 0–1 engineering agenda. **HYPOTHESIS**



Technology outlook: foundational, commodity, or Kriya's to own?

TECHNOLOGY	DIRECTION TO 2030	NATURE	KRIYA STANCE
Foundation and reasoning models	Cheaper and more capable every year	Commodity utility	INTEGRATE through the router
Multimodal / OCR	Near-human on scans and handwriting	Commodity	INTEGRATE ; keep deterministic parsers for known templates
Voice agents (Indic)	Real-time, telephony-grade; Meta Business Calling on WhatsApp	Commoditising	PARTNER (e.g. Sarvam)
Computer-use / browser agents	Rapid gains; UiPath Screen Agent ranked #1 on OSWorld-Verified (Jan 2026) [S17]	Commoditising	INTEGRATE models; BUILD verification wrappers
Memory and long context	Standard platform features	Commodity	INTEGRATE
Retrieval and knowledge graphs	Tooling commoditises; domain ontologies do not	Mixed	BUILD health and claims ontologies
Workflow engines and durable execution	Converge with agent runtimes	Infrastructure	BUILD thin policy layer; evaluate open-source engines
RPA	Absorbed into agentic automation suites	Incumbent-owned	AVOID competing; partner where installed
AI-native ERP / CRM	Suites embed agents; about \$0.10 per action list price [S15]	Incumbent-owned	INTEGRATE ; operate across them
Multi-agent orchestration	Every platform will offer it	Commoditising	BUILD thin (Omnitask); differentiate on policy and proof
Agent identity	Microsoft Entra Agent ID gives agents first-class directory identities [S19]	Foundational, IdP-owned	INTEGRATE IdPs; BUILD delegated-authority mapping
Agent payment authority	NPCI Unified Agent Protocol; Pine Labs P3P; card-network agent frameworks [S7]	Foundational, rail-owned	INTEGRATE rails; BUILD Mandate policies
Agent-to-agent protocols (MCP, A2A)	Standardised under Linux Foundation [S9]	Foundational standard	ADOPT natively
Agent observability	Traces and cost tooling commoditise	Mixed	BUILD business-outcome observability
AI governance and assurance	Regulation-driven (EU AI Act 2027–28; India sectoral) [S18]	Foundational, under-served	BUILD : Kriya's core
Physical AI, robotics, IoT, edge	Grows in hospitals and logistics after 2030	Specialist	WATCH ; ingest device events later; on-prem/edge for sovereign deployments

"Direction to 2030" entries are hypotheses except where sourced.

WHERE DURABLE ADVANTAGE CAN COME FROM

Only three kinds of layer can be both foundational and owned by a company Kriya's size: **(1) governance and assurance**, **(2) delegated authority policy** on top of rails others own, and **(3) domain ontologies with outcome data** in a value chain Kriya dominates. Everything else is integrated.

HYPOTHESIS



06

SECTION 06

2026 Market Reality

FACT Agent spend: measurement matters

Standalone “agentic AI” market estimates cluster at \$7–8.5B for 2025–26 (MarketsandMarkets, Precedence, Fortune BI, Deloitte). Gartner’s broader measure, which counts agentic capabilities embedded across software, is \$201.9B for 2026. That is a ~25x gap from definitions, not disagreement. [S16]

→ **Implication:** Don’t pitch TAM. Pitch a budget line the buyer already owns.

FACT The winners so far are incumbents

Salesforce reports Agentforce ARR above \$1.5B, with list pricing around \$0.10 per standard action and \$2 per resolved help-agent conversation. [S15]

→ **Implication:** Action-based pricing is normalising, and the price anchor is low. Kriya must price on outcomes and assurance, not on actions.

FORECAST Hypo correction is coming

Gartner forecasts more than 40% of agentic projects cancelled by end-2027 and estimates only ~130 “real” agentic vendors. [S1]

→ **Implication:** Survivors will show measured ROI and controls.

FACT India is moving to production

EY-CII: 47% of Indian enterprises have multiple GenAI use cases live; 91% say speed of deployment drives buy-vs-build. [S2]

→ **Implication:** Pre-built packs beat blank platforms.

FACT Legacy and governance are the blockers

OutSystems 2026: 38% say legacy systems stalled agentic projects; 36% have centralised agent governance. [S3]

→ **Implication:** Governance is a buying reason, not a feature.

FACT The channel owner became a competitor

Meta Business Agent is available globally (June 2026). An enterprise Business Agent Platform launched in India in July 2026. [S14]

→ **Implication:** Kriya cannot be “the WhatsApp bot”.

FACT The channel narrowed

General-purpose AI chatbots are banned from the WhatsApp Business API from 15 January 2026. [S8]

→ **Implication:** Purpose-bound design is mandatory.

FACT Execution is getting easier

Computer-use agents: UiPath’s Screen Agent ranked #1 on OSWorld-Verified (Jan 2026). Practitioners still report failures on authentication, anti-bot measures and long tasks. [S17]

→ **Implication:** Clicking commoditises. Verified outcomes don’t.

MARKET-SIZE DISCIPLINE

No TAM figure in this document should be read as Kriya’s addressable opportunity. The addressable opportunity is **specific budgets**: hospital revenue-cycle staff, TPA processing cost, employer benefits administration, DPDP compliance spend. Their sizes in India are **“Evidence insufficient”** in public sources today. Measuring them is a Phase 0 task, through buyer interviews.



07

SECTION 07

India Opportunity

93.95 cr

ABHA accounts; 105 crore linked health records; 5.33 lakh facilities registered (Jul 2026) [S21]

2,117

GCCs employing 2.36M people; Hyderabad drew nearly half of new BFSI GCCs [S4]

8.84 cr

MSMEs registered on Udyam platforms (30 Jun 2026) [S20]

₹15k/mo

expected full-delegation ceiling for AI agents paying on UPI under current UPI Circle rules [S7]

Where Indian organisations bleed, and what Kriya can do

SEGMENT	HOW WORK FLOWS TODAY	FINANCIAL CONSEQUENCE	KRIYA INTERVENTION
Hospitals & labs	WhatsApp, phone and paper glue between HMS/LIS, patients and payers	Front-desk load; report pickups; claim document chase	Kriya Health + Claims
Health insurance & TPAs	Portals, emails, PDFs; TPAs handle ~72% of cashless claims [S6]	Processing cost (NHCX aims ~₹500 → ~₹20 per claim)	Claims coordination + Proof
Large employers & GCCs	Benefits run through brokers, TPAs and HR inboxes	Renewal pricing with no population insight	Workforce Health (Lens)
Every data fiduciary	Consent and rights requests handled manually	DPDP obligations from May 2027 [S5]	Kriya Comply
MSMEs (8.84 crore on Udyam) [S20]	WhatsApp, Excel, GST and portal filings	Collections leakage; compliance workload	Via CA networks and partners, not direct
NBFCs & lenders	Document collection, KYC refresh, collections calls	Cost-to-collect; audit exposure	Document + Proof (not voice)
OEMs & dealers	Warranty claims via portals, photos on WhatsApp	Slow reimbursement; fraud	Claims pattern reuse (Phase 2)
Government-facing operations	Portals and grievance systems	Long cycles; tender-driven	Through system integrators only

BUILD SPECIFICALLY FOR INDIA

- WhatsApp-first, purpose-bound flows with voice fallback
- UPI mandates and agent-payment protocols
- ABDM/NHCX, GST and MCA portal connectors
- DPDP consent and rights operations
- Indian-language support through partners

GLOBALLY REUSABLE FROM DAY ONE

- Verified Action primitive, Proof, Mandate policy model
- Firewall framework (rules swap per jurisdiction)
- Zero-retention document intelligence
- MCP/A2A interfaces; model router
- Multi-party claims workflow template



08

SECTION 08

Global Opportunity

MARKET	ARCHITECTURE FIT	WILLINGNESS TO PAY	REGULATORY BARRIERS	LOCALISATION	ENTRY MODE	TIMING
Gulf (UAE, Saudi Arabia)	High: WhatsApp-heavy, Indian-linked hospital groups	High	Health data localisation; licensing	Arabic, English; payer rules	Partner with Indian hospital groups and TPAs present there	First , Phase 3
Southeast Asia (Indonesia, Philippines)	High: WhatsApp dominant in Indonesia [S22]	Medium	Local data laws	Bahasa; local payers	Local partner or distributor	Second, Phase 3–4
Africa (Nigeria, Kenya)	High channel fit	Low–Medium	Varies	Moderate	Partner-led; donor or insurer programmes	Phase 4, opportunistic
Latin America (Brazil)	High: WhatsApp ubiquitous [S22]	Medium	LGPD	Portuguese	Partner	Phase 4–5
United Kingdom	Medium	High	NHS procurement; UK GDPR	Low	Partner via health-tech suppliers	Phase 4+
European Union	Medium	High	EU AI Act high-risk rules from Dec 2027 / Aug 2028 [S18]; GDPR	High	Only with a compliance partner	Avoid before Phase 4
United States	Low (SMS/app-first)	Very high	HIPAA; state AI laws; liability	High	Would need US entity and team	Avoid before Phase 5
Australia	Low–Medium	High	Privacy Act; health regulation	Low	Partner	Phase 5, if at all

Gulf and Southeast Asia assessments are hypotheses built on channel dominance and Indian-linked health networks. Validate them with partner conversations before committing.

PROVE IN INDIA BEFORE ANY EXPANSION

- 3+ enterprise customers with contract values above ₹10 L and published, measured outcomes
- Proof receipts accepted by at least one auditor, insurer or TPA
- A cross-organisation workflow running at volume (claims or benefits)
- Gross margin above 65% without founder-led delivery
- Security posture that passes enterprise reviews (ISO 27001 path, VAPT, DPA)

WHY NOT THE US OR EU EARLY

- Highest willingness to pay, but the highest cost of sale and liability exposure
- Channel mismatch: WhatsApp is not the default in the US
- EU high-risk AI obligations add conformity work for health and insurance use cases
- Well-funded local competitors with established distribution
- Expanding there to look impressive is the classic premature-scaling failure

GLOBAL-BY-DESIGN PRINCIPLE

Keep jurisdiction-specific pieces (firewall rules, consent formats, payment rails, connectors) as swappable **policy and connector packs**. That way entering a new country means adding configuration, not forking the codebase.

KRIYA SCENARIO



09 SECTION 09 Industry Opportunity Map

Thirty industries were evaluated. The table has the 16 with any realistic fit; the remaining 14 are on the next page. **Strategic fit** means reuse of Kriya's durable assets plus a multi-party, regulated, document-and-money workflow. All ratings are my judgment **HYPOTHESIS**.

INDUSTRY	HIGHEST-VALUE LAST-MILE GAP	COMPETITIVE PRESSURE	DATA SENSITIVITY	SALES CYCLE	FIT	PRIORITY
Healthcare providers	OPD/IPD coordination, reports, deposits, discharge across HMS + WhatsApp	Med (HMS vendors; Meta at SMB end)	Very high	Med	Very high	P1
Diagnostics	Report delivery, corporate camps, B2B collections from referring clinics	Med	Very high	Short-Med	Very high	P1
Health insurance & TPAs	Claim document assembly, query loops, deductions, reconciliation	Med	Very high	Long	High	P1
Employer HR / benefits	Population health for renewals; employee claim help	Low-Med	High	Med	High	P1
Pharmaceuticals	Patient support, adherence, adverse-event intake for human review	Med	Very high	Long	Med-High	P2
General insurance (motor, property)	Claim intake, surveyor coordination, document proof	Med	High	Long	Med-High	P2
Professional services (CA/CS)	Client document chasing, compliance calendars, filing proof	Med	High	Short-Med	Med-High	P2
NBFC / financial services	Document collection, servicing, collections audit trail	Very high (voice-AI vendors)	High	Med	Med	P2
Automotive	Dealer warranty claims, service booking, parts disputes	Med	Med	Long	Med	P2
Human resources (enterprise)	Onboarding documents, verification, grievance intake	High (HRMS adding agents)	High	Med	Med	P2
Banking	Operations exceptions, KYC refresh, disputes	Very high (in-house, IT majors)	Very high	Very long	Med	P3
Manufacturing	Dealer/distributor orders, service tickets on WhatsApp	Med	Med	Long	Med	P3
Logistics	Exceptions: proof of delivery, delays, damage claims	Med	Low-Med	Med	Med	P3
Supply chain / procurement	Supplier onboarding documents, PO follow-ups	Med-High (suites)	Med	Long	Med	P3
Universities	Admissions, fee collection, student services	Med	Med (minors)	Long	Low-Med	P3
Energy / utilities	Connections, outage communications, billing disputes	Med	Med	Very long (PSUs)	Med	P3



The remaining 14 industries: why they are not priorities

INDUSTRY	GAP	PRESSURE	SENSITIVITY	VERDICT	WHY
Retail	Store operations, customer support	Very high	Low	AVOID	Commodity customer service; Meta and CPaaS own it
E-commerce / D2C	Support, returns, cash-on-delivery confirmation	Very high	Low	AVOID	Meta Business Agent's home turf [S14]
Education (schools, coaching)	Admission queries	High	Med (minors)	TEMPLATE	Seasonal, low willingness to pay
Hospitality	Reservations	High (OTAs)	Low	AVOID	Booking platforms own the customer
Travel	Itinerary changes, refunds	High	Low-Med	AVOID	Thin margins; incumbents
Real estate	Site visits, payment schedules, documents	Med	Med	TEMPLATE	Low regulation, so low trust premium
Construction	Subcontractor bills, safety compliance	Low	Low-Med	WATCH	Real pain, weak digital base; revisit 2030
Telecom	Enterprise onboarding, KYC	Very high (in-house)	High	AVOID	Builds its own; no wedge
Legal practice	Intake, conflict checks, deadlines	Med	Very high	TEMPLATE	Small firms, low budgets; bar-rule limits
BPO / KPO	Their own automation	High	Varies	PARTNER	Better as delivery channel than customer
Enterprise IT	IT service management	Very high (ServiceNow)	Med	AVOID	Incumbent-owned
Security	Security operations agents	Very high	Very high	AVOID	Specialist market; secure your own agents instead
Agriculture	Advisory, input credit, FPO coordination	Med (government-funded)	Low-Med	WATCH	Low willingness to pay; tender-driven
Government-facing operations	Citizen services, grievances	Med	High	VIA SI	Only through system integrators, from Phase 3

The pattern behind the map

KRIYA WINS WHERE ALL FIVE HOLD

- **Multi-party:** 3+ organisations must agree on one outcome
- **Regulated:** a regulator or auditor will ask "prove it"
- **Documents plus money move together** (claims, benefits, collections)
- **The citizen's channel is WhatsApp or voice**, not the enterprise app
- **Incumbents are strong inside one company but weak across companies**

KRIYA LOSES WHERE ANY OF THESE HOLD

- Single-company workflows inside a suite (IT, CRM, ERP tasks)
- Pure customer conversation (retail, e-commerce, travel)
- Low willingness to pay or tender-only buying (agriculture, schools)
- Buyers who build in-house by default (telecom, top banks)
- No trust premium: nobody needs proof



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SECTION 10

Enterprise Pain-Point Analysis

The objective is not to replace SAP, Salesforce or the HMS. It is to **operate intelligently across the systems enterprises already own**, in the human glue between them.



WORKFLOW	SYSTEMS TODAY	WHERE HUMANS FILL GAPS	FINANCIAL CONSEQUENCE	KRIYA INTERVENTION	ROI METRIC
Hospital cashless claim	HMS → TPA/insurer portal → email/WhatsApp → HMS billing	Billing staff re-key data, chase queries, upload documents	Staff hours; deductions and denials; slow receivables	Claims agent assembles, submits, chases, reconciles with Proof	Deduction rate; query cycles per claim; hours per claim
Corporate health camp	Lab LIS PDFs → Excel → HR slides	Manual transcription of hundreds of reports	Days of work; DPDP exposure	Kriya Lens batch ingestion, cohort dashboards	Turnaround; zero PII retained
Employee benefits help	HRMS → broker → TPA → employee WhatsApp	HR and broker answer calls and chase status	Employee dissatisfaction; weak renewal data	Workforce Health concierge + analytics	Tickets deflected; claim turnaround
B2B lab collections	Referral logs → invoices → reminder calls → UPI	Accounts team calls clinics	Days sales outstanding; leakage	Collections agent + Mandate	DSO; recovered ₹
DPDP rights request	Email/web form → legal → 5-10 systems	Legal searches systems by hand	Penalty exposure; slow response	Comply agent with erasure proofs	Response time; proof completeness
Dealer warranty claim	Dealer portal → OEM → Excel → photos on WhatsApp	Re-keying; photo chasing	Slow reimbursement; fraud	Claims template reuse + Proof	Cycle time; fraud flags
Vendor onboarding	Email → PDFs → ERP vendor master	Procurement staff collect and check documents	Onboarding delay; compliance gaps	Document agent + Proof	Days to onboard
Clinic revenue leakage	HMS ↔ payments ↔ billing	Nobody reconciles it	Unseen lost revenue	Leakage detection (planned)	Recovered ₹ per month

WHERE THE HIDDEN BUDGET SITS HYPOTHESIS

In people, not software lines. Revenue-cycle desks, TPA processing teams, benefits administration, compliance and legal operations. These costs appear as **headcount and outsourcing**, which is why buyers often don't list them as "AI budget". Kriya should sell against the headcount line, priced as a share of measured savings.



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SECTION 11

Competitive Landscape

Compare strategic positions, not logos. Named company capabilities are limited to what the sources support.

CATEGORY	WHAT THEY OWN	LIKELY TO BUILD NEXT HYPOTHESIS	KRIYA STANCE
Frontier labs (OpenAI, Anthropic, Google)	Models; computer use; consumer and enterprise agents	Vertical agent offerings through partners	Integrate. Never compete on intelligence.
Hyperscalers & identity (Microsoft, AWS, Google Cloud; Okta)	Cloud; agent identity (Entra Agent ID) [S19]; marketplaces	Agent registries, policy, governance within their estates	Integrate identity; list on marketplaces from Phase 2.
Enterprise suites (Salesforce, SAP, Oracle)	Systems of record; in-app agents (Agentforce ARR \$1.5B+) [S15]	Industry clouds including health and insurance	Don't fight inside their apps; operate across them.
Workflow / ITSM (ServiceNow)	IT and employee workflows	Cross-enterprise workflow agents	Avoid IT; integrate for escalations.
RPA / agentic automation (UiPath)	UI automation estates in large enterprises; Screen Agent #1 on OSWorld-Verified [S17]	Agentic process orchestration	Partner where installed; don't sell "bots".
Meta (WhatsApp)	The channel plus a native Business Agent; enterprise agent platform [S14]	Deeper commerce, booking and payments	Be the system behind the chat, not the chat.
Conversational AI and CPaaS (Gupshup, Yellow.ai, Haptik, Sarvam)	Channel operations; Indic voice; enterprise customer experience [S10]	Vertical packs	Partner for voice and Indic; compete only on proof and domain.
HMS / LIS vendors and health IT	Clinical and billing records	WhatsApp and AI add-on modules	Partner as the action layer; sell to their customers.
IT services majors (TCS, Infosys, Wipro, HCL, Tech Mahindra)	Enterprise relationships; implementation	Agent practices and accelerators	Channel partners from Phase 3.
AI-native vertical startups (revenue cycle, claims AI)	Point solutions	Adjacent expansion	Differentiate on cross-organisation work and proof; watch for acquisitions.

THE STRATEGIC OPENING

No category above natively owns **regulated, cross-organisation actions that carry proof**, running on India's public digital rails (ABDM, NHCX, UPI, DPDP consent) and on the citizen's channel. Each would have to partner to do it: suites are single-tenant by design, Meta avoids liability for regulated outcomes, and labs avoid domain liability. That is where Kriya should stand. **HYPOTHESIS**

WHAT COMPETITORS CAN COPY WITHIN 12 MONTHS

WhatsApp booking flows, multilingual support, dashboards, orchestration features, and connector-based integrations. **What they cannot copy quickly:** accumulated outcome data, accepted Proof formats, signed multi-party integrations, and trust relationships with regulated buyers.



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SECTION 12

Kriya Category Strategy

Nine candidate identities, evaluated on differentiation, defensibility, buyer clarity and feasibility.

CANDIDATE CATEGORY	DIFFERENTIATION	DEFENSIBILITY	BUYER	VERDICT
A · AI operating layer	Low: everyone claims it	Low	Unclear buyer	Reject
B · Autonomous enterprise workforce	Medium	Low: suites ship “digital workers”	HR/COO, vague	Reject; later narrative
C · Agentic operations platform	Low–Medium	Low	COO	Reject: crowded
D · AI execution layer	Medium	Medium: execution commoditises	CIO	Partial: execution alone isn’t enough
E · Enterprise agent infrastructure	Low	Low: hyperscalers own it	CIO/CTO	Reject
F · Autonomous business–process platform	Medium	Medium	COO/CFO	Partial
G · AI employee operating system	Low	Low	HR	Reject: persona framing ages badly
H · Industry-specific autonomous OS	High in one vertical	High, but caps the ceiling	Vertical ops head	Use as the entry form, not the end-state
I · Verified Action Infrastructure	High: nobody owns “provable cross-organisation action”	High: data, proof formats, multi-party integrations	COO/CFO now; CRO, CISO and regulators later	Select

THE CATEGORY KRIYA SHOULD CREATE

Verified Action Infrastructure: systems that execute consequential business actions across organisations under delegated authority. Every action is policy-checked, verified in the target system, and backed by a receipt that other parties can trust.

Positioning line: “Kriya: verified action.” **Promise:** “AI that acts, and proves it acted right.” **Entry form:** an industry-specific autonomous operating system for the healthcare value chain (option H), built on the category’s primitives (option I).

EVIDENCE THAT WOULD PROVE THE STRATEGY WRONG

1. Buyers say they would never pay for proof, only for cost reduction. Then Kriya becomes a vertical workflow company (Scenario A).
2. Insurers and TPAs refuse to accept third-party receipts even after a reference exists.
3. A suite or Meta ships a cross-organisation verification feature at near-zero price before 2029.



Stress-testing the category against the ten criteria

CRITERION	EVIDENCE FOR	WHAT COULD PROVE IT WRONG
1. Large enterprises actually need it	Claims, benefits and compliance run on human glue; governance is a top barrier [53]	Needs buyer interviews to confirm budget ownership
2. Existing software doesn't solve it	Suites act within one tenant; Meta avoids regulated outcomes	A suite could add cross-tenant "trust" features
3. Foundation models don't solve it alone	Authority, liability and multi-party agreement aren't model capabilities	Labs could partner with insurers directly
4. Kriya can realistically build it	Firewall, state machine, ledger, risk gates and zero-retention parsing exist	Enterprise-grade security and scale are unproven
5. Can begin in healthcare	Kriya already executes bookings, refunds and report delivery	Hospital sales cycles and Meta App Review
6. Can expand beyond healthcare	Claims and document patterns recur in insurance, auto warranty, lending	Each vertical needs domain partners
7. Has measurable ROI	Deductions, hours per claim, days sales outstanding, response times	No customer baselines measured yet
8. Supports enterprise pricing	Sold against headcount and leakage, not seats	The \$0.10-per-action anchor could compress prices
9. Can be globally relevant	Every jurisdiction regulates health, money and identity actions	Local regulation and incumbents differ
10. Survives technology change to 2040	Better models increase the need for authority and proof	A universal agent-trust standard owned by a giant

Kriya AI's unique category creation strategy



1 · Name the problem, not the product. "Unaccountable automation": agents that act without authority, verification or proof. Publish the cost of it in Indian healthcare (deductions, rework, disputes) using your own measured data.



2 · Win one undeniable reference. One hospital group or employer where the cross-organisation workflow runs on Kriya with published before/after numbers.



3 · Open the receipt format. Publish the Kriya Proof receipt as an open specification, so auditors, TPAs and insurers can verify receipts without buying Kriya. Standards spread categories.



4 · Recruit the other parties. Every insurer, TPA or employer that accepts Proof receipts makes Kriya more valuable to the next hospital. That is the network effect.



5 · Graduate to certifying others. Once the format is trusted, offer verification for third-party agents (Kriya Verify). The category becomes infrastructure.



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SECTION 13

Future Architecture

The layers from the brief, modified after research. Two layers were renamed (Identity → Identity & Authority; Security & Governance → Proof & Governance) because that is where Kriya can own something. Highlighted layers are the moat.

1	Interfaces Channels	WhatsApp, voice, SMS/RCS, email, web, mobile, APIs, and MCP/A2A endpoints so other agents can call Kriya actions	INTEGRATE
2	Identity & Authority Kriya Mandate	Human SSO and agent identity from IdPs; delegated authority (who may do what, up to what limit, until when) is Kriya's own model	OWN AUTHORITY
3	Intelligence Omnitask Router	Model-portable brain-body separation; routing by cost, latency, jurisdiction; bring-your-own key or managed	BUILD THIN
4	Agent Runtime Kriya Omnitask	Hierarchical orchestrator, typed contracts, retries, compensation, escalation chains	BUILD
5	Policy & Workflow Omnitask Policy Engine	Deterministic state machines and DAGs; firewall packs; risk tiers; human approval gates	OWN
6	Integration Fabric Kriya Reach	HMS/LIS, NHCX, insurer/TPA portals, ERP/HRMS, UPI; computer-use models for portals without APIs, wrapped in verification	BUILD + INTEGRATE
7	Proof & Governance Kriya Proof	Hash-chained receipts, signed exports, auditor views, retention policy, tenant isolation, encryption	OWN
8	Observability & Outcomes Kriya Outcomes	Traces, cost, latency, and above all business outcomes per workflow	BUILD OUTCOMES
9	Operational Intelligence Kriya Lens + Digital Twin	Zero-retention document intelligence; process twin; anomaly and leakage detection	OWN
10	Autonomous Optimisation Phase 3+	Learn from outcome data to re-plan workflows, balance load, and intervene predictively	LATER



Converging two codebases without a rewrite

Kriya Health (Python/FastAPI) and Kriya Omnitask (TypeScript-typed agents, React 19) should not be merged in one big-bang rewrite. That would freeze sales for months. Use a **strangler pattern** instead, as you already did with CallMedex's frozen-contract integration.

STEP	WHAT HAPPENS	WHEN KRIYA SCENARIO	DONE WHEN
1 • Contract	Expose Kriya Health capabilities (book, hold, refund, deliver report, parse batch, firewall check) as signed internal APIs with versioned schemas	Oct–Dec 2026	Omnitask can call every Health action through the contract
2 • Control plane	Omnitask becomes the orchestrator, console and policy layer for new workflows; Health keeps executing clinical actions	Jan–Jun 2027	New packs (Workforce Health) built only on Omnitask
3 • Shared trust	One tenant model, one Proof ledger, one Mandate service used by both	Apr–Sep 2027	Every consequential action in both systems writes a Proof receipt
4 • Absorb	Port Health modules into Omnitask only when touched for new work; retire duplicates	2028	No duplicate booking, payment or messaging logic remains

Architecture principles

Deterministic where it matters

Models propose; policies decide. Any action above risk tier 1 passes the policy engine, never a prompt.

Proof before confirmation

The receipt is written before the user or counterparty is told “done”.

Model- and channel-portable

No workflow may depend on one model provider or on WhatsApp alone.

Configuration, not code, per vertical

Packs are ontologies, policies, templates and connectors. Custom code per customer is a defect.

Sovereign by option

India-region hosting by default for enterprise; on-premise or private-cloud runtime by Phase 3.

Callable by other agents

Every Kriya action is also an MCP tool or A2A skill, gated by Mandate and recorded in Proof.

CONVERGENCE RISKS

1. **Two languages, one team of one.** Keep Python for clinical and parsing services and TypeScript for orchestration and consoles. Don't port for its own sake.
2. **Tenant model drift.** Define one canonical tenant, user and role schema before Step 3; migrate both systems to it.
3. **Hidden coupling.** Contract tests on every internal API, run in CI for both repositories.

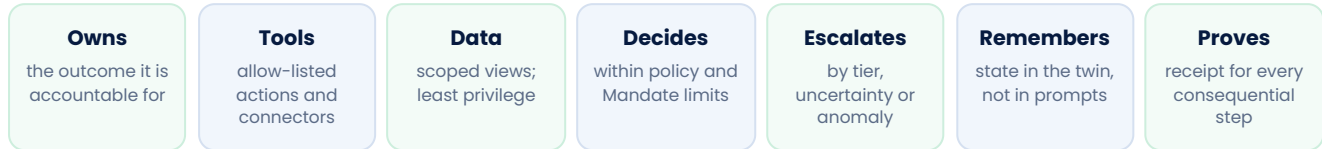


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SECTION 14

Agent Architecture

Every Kriya agent is defined by a **charter**, not a prompt. The charter is versioned, reviewed and enforced by the runtime.



Risk tiers (extends Omnitask's existing 4-tier gates)

TIER	SCOPE	EXECUTION	EXAMPLES
T0 · Inform	Read-only: status, reminders, summaries	Automatic	Report-ready notice; queue token
T1 · Reversible	Low-value, reversible actions within policy	Automatic + receipt	Book or reschedule a slot; send a document request
T2 · Consequential	Money or records move within Mandate limits	Automatic if the policy passes; sampled human review	Refund under ₹X; submit a complete claim packet
T3 · Irreversible or clinical	High-value, clinical, legal or identity actions	Human approval required	Anything touching diagnosis or dosage; large payments; erasure of records

Lifecycle of one verified action



Worked example: doctor on emergency leave

STEP	AGENT	TIER	PROOF WRITTEN
Leave marked in doctor portal	Orchestrator detects event	T0	Event receipt
Find affected appointments	Scheduling	T0	Affected-list hash
Cancel slots and release holds	Scheduling	T1	Cancellation receipts
Refund prepaid fees under limit	Payments & Mandate	T2	Refund receipts with gateway reference
Refund above Mandate limit	Attention → human approver	T3	Approval record
Offer one-tap rebooking on WhatsApp	Intake	T1	Message receipts
Verify refunds settled	Verification	T0	Settlement confirmation

DESIGN RULE THAT SEPARATES KRIYA FROM AGENT BUILDERS

If an action can't be verified in the target system, it is **not reported as done**. It is reported as "submitted, awaiting confirmation" and tracked until verified or escalated. Most agent products skip this, and it is where trust is lost.



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SECTION 15

Multi-Agent Strategy

AGENT	OWNS	TOOLS	DATA ACCESS	AUTO-EXECUTE	NEEDS A HUMAN FOR	PHASE
Intake / Concierge	First contact and intent capture	Channels, language models, ontology	Contact profile, consented history	T0-T1	None	0
Scheduling	Slots and resources	Calendars, HMS, slot holds	Resource tables	T1	Overbooking exceptions	0
Payments & Mandate	Collections, holds, refunds	Razorpay, UPI agent protocols	Ledger; Mandate limits	T2 within limits	Above limit; disputes	0-2
Document (Lens)	Extract and validate documents	Parsers, OCR models	In-memory only; structured output	T1	Low-confidence parses	0-1
Claims	Claim packet to settlement	HMS, NHCX, insurer/TPA portals	Claim, policy and billing data	T2	Denials; clinical justification	1-2
Collections	Receivables follow-up	Invoices, channels, UPI	Receivables ledger	T1-T2	Settlements and waivers	1
Consent & Compliance	DPDP rights and consent records	Consent ledger, system connectors	Personal-data map	T1-T2	Erasure; legal holds	1
Verification	Checks other agents' results	Read-back connectors	Read-only	T0	Mismatches	1
Attention / Escalation	Routes to the right human	Human Attention Center	Case context	T0	Always hands to a human	0
Outcome Analyst	Measures and recommends	Outcome store, twin	Aggregated, anonymised	T0	Changes to policy	2
Orchestrator	Plans, delegates, resolves conflicts	All agents via typed contracts	Task graph	Per-task tier	Unresolvable conflicts	0

How agents cooperate

Communication

Typed contracts (Omnitask's Zod schemas) between agents. No free-text hand-offs for consequential steps.

Single writer per resource

Only the Scheduling agent writes slots; only Payments writes the ledger. Others request, never write.

Conflict resolution

Precedence: **safety** → **compliance** → **the customer's explicit instruction** → **cost/efficiency**. Ties go to a human.

Shared memory

The Business Digital Twin holds state; each agent sees a scoped view. Prompts never carry the memory.

Organisation policies

Tenant policies override agent defaults; Mandate limits override both.

External agents

Other companies' agents enter through the Kriya Gateway over A2A/MCP. They get the same Mandate checks and the same Proof, with no side doors.

DON'T OVER-AGENT

Start with the **five agents Phase 0 needs** (Intake, Scheduling, Payments, Document, Attention). Every extra agent is extra failure surface. Add one only when a workflow can't be expressed as policy plus an existing agent.



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SECTION 16

Vertical Product Strategy

A pack is **configuration on the platform**: domain ontology + firewall rules + policies + connectors + templates + KPI definitions. It is not a new product codebase.

PACK	BUYER	WHAT IT IS	WHEN KRIYA SCENARIO
Kriya Health	Providers & labs	Today's product, re-based on Omnitask	Now
Kriya Workforce Health	Employers, GCCs, brokers	Lens + employee concierge + renewal pack	Phase 0–1
Kriya Claims	Hospitals (revenue cycle), TPAs, insurers	Claims agent + Reach + Proof	Phase 1
Kriya Comply	Any data fiduciary	Rights desk, consent ledger, erasure proofs	Phase 1 add-on
Kriya Pharma	Pharma companies	Patient support; adverse-event intake for human review	Phase 2
Kriya Claims+ (warranty, general insurance)	OEMs, dealers, general insurers	Reuse claims template with new ontology	Phase 2–3
Kriya Professional	CA/CS networks, mid-size companies	Compliance calendar, document chase, filing proof (absorbs the GST copilot idea)	Phase 2–3
Kriya Verify	Any enterprise running agents	Certification and runtime policy for third-party agents	Phase 4

GATE FOR LAUNCHING A NEW PACK

- 3 paying design partners signed **before** building
- The majority of the pack reuses existing platform components (measure it; don't assume)
- No new channel dependency
- A named domain advisor
- The previous pack has hit its retention target

SIGNALS TO STOP A PACK

- Custom work per customer above 30% of pack effort
- Sales cycle double the plan with no conversions
- Gross margin below target after 12 months
- Trust premium absent: buyers don't value Proof

WHY THIS ORDER

Each pack reuses the previous pack's **counterparties**. Hospitals bring labs and TPAs. Labs bring employers. Employers bring brokers and insurers. Insurers bring general insurance and warranty claims. Expansion follows relationships and document flows, not a list of sectors. **HYPOTHESIS**



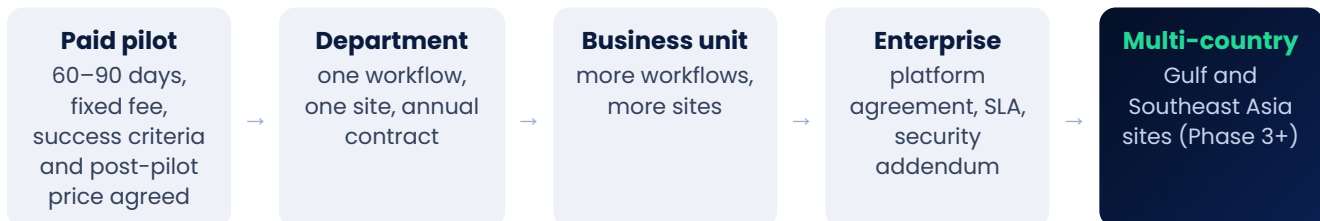
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SECTION 17

Corporate Go-To-Market

BUYER	PROBLEM THAT GETS BUDGET	OUTCOME THAT JUSTIFIES PURCHASE	ENTRY OFFER
Hospital CFO / revenue-cycle head	Claim deductions, receivables days, desk staff cost	Deduction %; days sales outstanding; hours per claim	Claims pilot on one payer
COO / operations head (hospital or lab chain)	Turnaround, front-desk load, report pickups	Turnaround; no-show %; staff hours	Kriya Health enterprise
CHRO / Total Rewards head	Group-health renewal cost; employee experience	Screening coverage; cohort risk shift; tickets deflected	Workforce Health via the lab
Head of claims (TPA / insurer)	Cost per claim; query cycles	Cost and time per claim	Proof-accepted claim channel
DPO / CISO / General Counsel	DPDP obligations; agent governance	Rights-request time; audit completeness	Comply add-on
Diagnostics business-unit head	B2B revenue; corporate camp margins	Camp turnaround; B2B collections	Corporate Health Insights
CIO / CTO	Integration risk; security; vendor count	Uptime; incidents; integration effort	Security pack and architecture review

Land and expand



WHAT PROCUREMENT WILL ASK FOR

- Security questionnaire; VAPT report; ISO 27001 roadmap, then certification
- Data processing agreement; India data residency; sub-processor list
- SLA: uptime, support hours, response and resolution times
- Incident response and breach-notification plan aligned to DPDP
- Cyber and professional-indemnity insurance
- Audit rights, exit plan, data portability

CHANNELS THAT SHORTEN THE CYCLE

- **Diagnostic labs** (the Taiyo model) into employers
- **Group-insurance brokers** into large employers
- **HMS/LIS vendors** into hospitals, as an add-on
- **TPAs** into their hospital networks
- **System integrators** into BFSI and government (Phase 3)



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SECTION 18

Commercial Model

MODEL	BEST FIT	STAGE	RISK / NOTE
SaaS subscription (tiers)	SMB clinics and labs	Phase 0 only	Meta Business Agent compresses the value of basic booking
Annual platform licence	Chains, employers, TPAs	Phase 1+	Needs clear scope to avoid unbounded custom work
Per verified action	High-volume workflows (claims steps, refunds)	Phase 1+	Anchored against suite pricing (about \$0.10 per action) [S15]
Per workflow / per case	Claims, rights requests, onboarding	Phase 1+	Simple for buyers; needs mix management
Per seat	Rarely suitable	Avoid	Punishes automation success
Per agent	Marketing-friendly only	Avoid	Doesn't map to value
Managed AI operations	Enterprises wanting outcomes without running it	Phase 1–3	Services margin trap if not automated
Outcome-based (share of savings)	Deductions reduced, receivables recovered	Phase 2+	Requires trusted baselines and Proof
Hybrid (platform + usage + outcome kicker)	Enterprise default	Phase 1+	Complex contracts; needs a finance lead

Pricing principles

Price the outcome, not the tokens Anchor on the headcount and leakage the buyer already pays for. Aim for a price that is a clear minority of the measured saving. ' + H + '

Charge for trust separately Proof exports, auditor access and extended retention are premium features. Assurance is what competitors can't match on price.

Setup fees, always Every enterprise onboarding carries a fee. It filters out unserious pilots and funds implementation.

Keep the bring-your-own-key option Omnitask's BYO model key suits tech-savvy mid-market buyers. Regulated enterprises will prefer **Kriya-managed inference** with India residency, priced with margin.

DECISION ON THE CURRENT SMB TIERS **HYPOTHESIS**

Meta's native agent now does basic booking and lead qualification **for free inside WhatsApp** [S14]. Don't discount to compete. Reposition the tiers around what Meta doesn't do: HMS/LIS integration, report delivery, payments with slot holds and refunds, clinical firewall, multi-branch operations and Proof. Then let the lowest tier fade if it stops converting.



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SECTION 19

Brand Transformation

MASTER BRAND

KRIYA

Verified action. AI that acts, and proves it acted right.

THE ENGINE

Kriya Omnitask

Autonomous operations runtime: orchestration, policy, agents

TRUST

Kriya Proof

receipts and audit

AUTHORITY

Kriya Mandate

delegated limits

INTELLIGENCE

Kriya Lens

zero-retention insight

REACH

Kriya Reach

connectors

FLAGSHIP
SOLUTION

Kriya Health

SOLUTION
Workforce HealthSOLUTION
Kriya ClaimsSOLUTION
Kriya Comply

BRAND ATTRIBUTE	CARRIED BY	PROOF POINT CUSTOMERS CAN CHECK
ACTION	"Kriya" (Sanskrit for action)	Completed bookings, claims, refunds
EXECUTION	Kriya Omnitask	Task completion and verification rates
INTELLIGENCE	Kriya Lens	Insight without data custody
AUTONOMY	Omnitask risk tiers	Share of actions executed without humans, safely
TRUST	Kriya Proof	Receipts verified by third parties
ENTERPRISE RELIABILITY	SLA, certifications	Published uptime; audits passed

CHANGES TO MAKE NOW

- Healthcare becomes the **flagship solution**, not the master identity
- Replace "Hospital Operating System" on the logo lockup with "Verified action"
- Remove "Engineered by Xylarc AI" from the product lockup, per the Omnitask decision
- Keep the K device: it already reads as structure plus motion
- One domain and one email family for Kriya (entity decision pending)

NAME RISK (UNCHANGED)

- "Kriya" is crowded: a UK fintech (Allica Bank), Kriya Inc (AI scheduling, US), Kriya Nextwealth (Bengaluru), Kriya Design Technologies (Hyderabad health tech), Criya AI (YC) [511]
- A trademark search and filing by an attorney must precede enterprise marketing
- Protect the "Kriya AI" wordmark plus the K device; consider "Kriya Omnitask" as a separate mark



Roadmap overview: six phases to 2040

The timeline was adjusted from the brief's placeholders. Phase 0 runs to graduation (2027), and later phases are longer, because enterprise adoption and regulation move slower than technology. **Gates beat dates.** Each phase must be a viable business even if the next never happens.

PHASE 0 Foundation & healthcare proof Oct 2026 – Jun 2027	PHASE 1 Platform extraction & adjacent verticals Jul 2027 – Jun 2029	PHASE 2 Cross- boundary network Jul 2029 – 2031	PHASE 3 Autonomous regulated operations 2031 – 2034	PHASE 4 Agent verification ecosystem 2034 – 2037	PHASE 5 Global verified- action infrastructure 2037 – 2040
Prove Kriya Health; converge onto Omnitask; first corporate pilot	Workforce Health, Claims, Comply on Omnitask	Multi-party health network; Mandate on UPI; second value chain	End-to-end autonomous back-office workflows; first international	Kriya Verify for third-party agents; Proof as a standard	Cross-border agent-to-agent assurance in regulated sectors
Gate 8–12 paying tenants, 1 enterprise LOI	ARR target ₹2–5 Cr	ARR target ₹25–75 Cr	ARR target ₹250 Cr+	Signal Proof named by regulators or payers	Endgame IPO / strategic / DPI partner
Team 1–3	Team 8–15	Team 40–100	Team 150–300	Team 400–800	Team Scenario-dependent

HOW TO READ THE PHASE PAGES

Each phase page covers the brief's required fields: objective, thesis, products, industries and customers, architecture and agents, integrations and partnerships, team and capital, go-to-market and pricing, geography and regulation, moat, KPIs, risks, and exit criteria. ARR ranges are **internal targets** **KRIYA SCENARIO**, not forecasts.

Milestone spine, 2026–2040

WHEN	MILESTONE KRIYA SCENARIO	WHAT IT PROVES
Dec 2026	Proof v0 live; Taiyo pilot live; entity and brand decided	Thesis is buildable
Jun 2027	Phase 0 gates; founder full-time	Healthcare demand is real
2028	First ₹10 L+ enterprise contract; ISO 27001; seed	Enterprises will buy
2029	A TPA or insurer accepts Kriya Proof receipts	Counterparties value proof
2031	Cross-organisation volume ≥30%; second value chain live	Network effect exists
2034	First international revenue; autonomous operations under SLA	Model travels
2037	Kriya Verify certifying third-party agents	Kriya is infrastructure
2040	Proof and Verify embedded in regulated cross-border workflows	Category leadership



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SECTION 20

Phase 0 · Foundation & Healthcare Proof

PHASE
0

Foundation & Healthcare Proof

OCTOBER 2026 – JUNE 2027 · THROUGH GRADUATION

Objective: turn engineering depth into market proof. **Thesis:** one undeniable healthcare reference plus a converged platform are worth more than any new sector.

PRODUCTS

- Kriya Health, repositioned beyond basic booking
- Corporate Health Insights (Taiyo pilot)
- Proof receipts v0 on bookings, refunds and reports
- Kriya Omnitask as control plane (Step 1–2)

INDUSTRIES & CUSTOMERS

- Clinics, polyclinics, diagnostic centres (AP/Telangana)
- One diagnostic chain plus its first corporate client
- Discovery only: employers, hospital revenue cycle, TPAs

ARCHITECTURE & AGENTS

- Contract layer between Health and Omnitask
- 5 agents: Intake, Scheduling, Payments, Document, Attention
- Second LLM provider fallback
- Outcome instrumentation per tenant

INTEGRATIONS & PARTNERSHIPS

- MocDoc, Razorpay, WhatsApp Cloud API (Meta review)
- Lab partner (Taiyo); 1 HMS vendor conversation

TEAM & CAPITAL

- Founder + 1–2 part-time; bootstrapped; cloud and model credits
- Search for a commercial cofounder or senior hire starts

GO-TO-MARKET & PRICING

- Founder-led sales; existing tiers repositioned
- Paid corporate pilot with success criteria

GEOGRAPHY & REGULATION

- AP/Telangana; Hyderabad for employer interviews
- DPDP readiness before May 2027; claim hygiene in all documents

MOAT BUILT

- First measured outcome data
- First Proof receipts in live traffic

KPIs

- Paying tenants; logo retention
- Measured no-show and turnaround change
- Interviews completed; LOIs

MAJOR RISKS

- Founder bandwidth (degree plus multiple projects)
- Meta review delays
- Meta Business Agent pressure on low tiers

EXIT CRITERIA → PHASE 1

- ≥8 paying tenants, ≥90% monthly logo retention
- 2 case studies with measured numbers
- 1 enterprise LOI; brand, entity and trademark decided

KILL / PIVOT

- <5 paying by Mar 2027: fix ICP and pricing before building anything new
- No LOI from 20 interviews: re-rank the wedges



SECTION 21

21

Phase 1 · Platform Extraction & First Adjacent Verticals

PHASE

1

Platform Extraction & First Adjacent Verticals

JULY 2027 – JUNE 2029

Objective: become the verified action layer for healthcare enterprises and the corporates they serve. **Thesis:** adjacent packs sharing counterparties grow faster than new sectors.

PRODUCTS

- Workforce Health; Kriya Claims (hospital revenue cycle first); Comply add-on
- Proof v1 (auditor export); Mandate v1; Lens generalised

INDUSTRIES & CUSTOMERS

- Hospital groups and diagnostic chains
- Employers (2,000+ staff) and GCCs via labs and brokers
- 1–2 TPAs as Proof acceptors

ARCHITECTURE & AGENTS

- Shared tenant model, Proof and Mandate across both codebases
- + Claims, Collections, Consent, Verification agents
- MCP server: Kriya actions callable by client agents

INTEGRATIONS & PARTNERSHIPS

- Top HMS/LIS; NHCX; insurer/TPA portals via Reach
- Group-insurance brokers; one HMS vendor alliance

TEAM & CAPITAL

- 8–15 people; enterprise sales and security leads
- Seed round after the Phase 0 gates (range in Section 30)

GO-TO-MARKET & PRICING

- Paid pilot → department → business unit
- Annual platform fee + per case; setup fees

GEOGRAPHY & REGULATION

- South India, then metro hospital groups
- ISO 27001 certification; DPDP full compliance; IRDAI and NHCX alignment

MOAT BUILT

- Health and claims ontologies with outcome data
- Proof accepted by a first counterparty

KPIs

- ARR; net revenue retention; gross margin
- Verified-action rate; escalation rate
- Deduction and turnaround improvements

MAJOR RISKS

- Enterprise cycles longer than planned
- Services trap (custom work)
- Security review failures

EXIT CRITERIA → PHASE 2

- ₹2–5 Cr ARR **KRIYA SCENARIO**; 3+ customers above ₹10 L contract value
- NRR >110%; one counterparty accepting Proof

KILL / PIVOT

- <2 enterprise wins in 12 months: go mid-market via HMS and lab channels
- Custom work >30%: stop sales and productise



SECTION 22

22

Phase 2 · Cross-Boundary Network & Multi-Vertical Platform

PHASE

2

Cross-Boundary Network & Multi-Vertical Platform

JULY 2029 – 2031

Objective: connect hospitals, TPAs, insurers, employers and patients on one verifiable fabric, then open a second value chain. **Thesis:** each accepting party raises Kriya's value to the next (network effect).

PRODUCTS

- Claims at network scale; Mandate on UPI agent protocols
- Kriya Pharma or Claims+ (chosen by data)
- Proof open specification v1

INDUSTRIES & CUSTOMERS

- Insurers, TPAs, hospital networks, large employers
- Pharma or OEM warranty programmes

ARCHITECTURE & AGENTS

- A2A gateway for counterparties' agents
- Outcome Analyst agent; policy recommendations with human approval
- India-region dedicated deployments

INTEGRATIONS & PARTNERSHIPS

- NPCI-ecosystem payment partners
- Voice and Indic partner; cloud marketplaces

TEAM & CAPITAL

- 40–100 people; insurance domain head; CFO; regulatory affairs
- Series A/B; possible strategic investor

GO-TO-MARKET & PRICING

- Network pricing: per verified action + platform
- Outcome kickers on deductions and recoveries

GEOGRAPHY & REGULATION

- Pan-India; Gulf partner discovery
- Audited financials; sector regulator engagement

MOAT BUILT

- Multi-party integrations; network effect
- Cross-organisation outcome benchmarks

KPIs

- Organisations connected; % cross-organisation volume
- Cost per verified action; dispute rate

MAJOR RISKS

- Insurers refuse third-party receipts
- Incumbent TPA or HMS software bundles similar features

EXIT CRITERIA → PHASE 3

- ₹25–75 Cr ARR **KRIYA SCENARIO**; gross margin >65%
- ≥3 insurer/TPA integrations; ≥30% cross-organisation volume

KILL / PIVOT

- No payer adoption: stay hospital-side (revenue cycle). Still a strong company (Scenario A).



SECTION 23

23

Phase 3 · Autonomous Operations for Regulated Enterprises

PHASE
3

Autonomous Operations for Regulated Enterprises

2031 – 2034

Objective: run end-to-end back-office workflows autonomously under policy and proof. This is the “autonomous enterprise workforce” in its safe form.

Thesis: by now enterprises buy operated outcomes, not tools.

PRODUCTS

- Autonomous claims, benefits and compliance operations with human oversight councils
- Sovereign/on-prem runtime
- Outcome guarantees in contracts

INDUSTRIES & CUSTOMERS

- Insurers, hospital groups, employers, pharma, general insurance, lenders (documents and proof)

ARCHITECTURE & AGENTS

- Autonomous optimisation layer live
- Agent identity federation with enterprise IdPs

INTEGRATIONS & PARTNERSHIPS

- System integrators as channels; insurers underwriting agent-action liability

TEAM & CAPITAL

- 150–300 people; Chief Trust Officer; international GMs
- Growth capital

GO-TO-MARKET & PRICING

- Enterprise agreements with outcome-based components
- Marketplace distribution

GEOGRAPHY & REGULATION

- First international market (Gulf) via partners
- EU AI Act awareness for any EU-facing work

MOAT BUILT

- Liability partnerships; long-horizon outcome data

KPIs

- Autonomous completion rate at SLA; incidents
- International revenue share

MAJOR RISKS

- A serious liability incident
- Giants ship a comparable trust layer

EXIT CRITERIA → PHASE 4

- ₹250 Cr+ ARR **KRIYA SCENARIO**
- Proof referenced by a regulator, payer council or auditor body

KILL / PIVOT

- Liability cannot be insured: cap autonomy at T2 and sell operated services



SECTION 24

24 Phase 4 · Agent Ecosystem & Verification Infrastructure

PHASE 4

Agent Ecosystem & Verification Infrastructure

2034 – 2037

Objective: verify and govern other companies' agents in regulated flows, not only Kriya's. **Thesis:** when every institution runs agents, the scarce role is the trusted verifier between them.

PRODUCTS

- Kriya Verify: certification, runtime policy enforcement, agent registry
- Proof as an industry format

INDUSTRIES & CUSTOMERS

- Any regulated enterprise with agents; regulators and auditors as users

ARCHITECTURE & AGENTS

- Verification network across tenants; privacy-preserving attestations

INTEGRATIONS & PARTNERSHIPS

- Standards bodies, auditors, insurers, payment rails

TEAM & CAPITAL

- 400–800 people; standards and policy team

GO-TO-MARKET & PRICING

- Certification subscriptions; per-verification fees; revenue share with assurance partners

GEOGRAPHY & REGULATION

- India + Gulf + Southeast Asia; selective Europe via partners

MOAT BUILT

- Being the accepted verifier: hardest position to displace

KPIS

- Third-party agents verified; receipts verified externally

MAJOR RISKS

- Standards fragmentation; neutrality questioned

EXIT CRITERIA → PHASE 5

- Kriya Proof or Verify named in payer, regulator or industry frameworks

KILL / PIVOT

- If a neutral public body owns verification, become its leading implementation partner



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SECTION 25

Phase 5 · Global Verified-Action Infrastructure

PHASE

5

Global Verified-Action Infrastructure

2037 – 2040

Objective: be the assurance and execution backbone for cross-institution, agent-to-agent work in regulated sectors across the Global South and selected developed markets. **Thesis:** trust infrastructure compounds like payment networks.

PRODUCTS

- Cross-border agent-to-agent assurance; outcome-operated back offices
- Consented, anonymised benchmarks and industry datasets

INDUSTRIES & CUSTOMERS

- Health, insurance, benefits, lending, compliance: the regulated core

ARCHITECTURE & AGENTS

- Federated, jurisdiction-aware verification; edge and sovereign nodes

INTEGRATIONS & PARTNERSHIPS

- Digital public infrastructure partners in several countries

TEAM & CAPITAL

- Scenario-dependent; public-market or strategic capital

GO-TO-MARKET & PRICING

- Outcome and network pricing; country partnerships

GEOGRAPHY & REGULATION

- Multi-region, with per-country policy packs

MOAT BUILT

- Network, standard, data and liability capacity together

KPIs

- Share of regulated cross-organisation actions verified in core markets

MAJOR RISKS

- Geopolitics and data-localisation fragmentation
- Platform consolidation by giants

ENDGAME OPTIONS

- IPO; strategic merger with a payments or insurance major; long-term DPI partner

HONEST NOTE

- Reaching this phase is low-probability. The plan is designed so Phases 1–3 are worth winning on their own.



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SECTION 26

Scenario Analysis

Three futures for Kriya itself. Not predictions: which one happens depends on evidence gathered at each gate.

	A · CONSERVATIVE	B · ENTERPRISE PLATFORM	C · BREAKTHROUGH
Definition	Kriya stays a strong vertical SaaS for the healthcare value chain (providers, labs, revenue cycle)	Kriya becomes a multi-industry verified-operations company across 4–6 regulated value chains	Kriya becomes infrastructure: the verifier for agent-to-agent action across institutions
What must go right	Health references; margin discipline; retention	Proof accepted by payers; a second value chain lands; a strong enterprise team	Regulation or payers demand agent assurance; neutral positioning; global partners
What can go wrong	Meta/HMS vendors bundle features; slow hospital cycles	Services trap; incumbents copy; capital gaps	Giants or public bodies own verification; liability shock
Investment (hypothesis)	Mostly bootstrapped plus seed	Seed + Series A/B	Multiple growth rounds; strategic capital
Technology requirements	Health connectors, Proof v1, Lens	Multi-tenant runtime, Mandate, A2A gateway	Verification network, sovereign deployments, attestations
Market requirements	Hospitals pay for integration and outcomes	Counterparties accept receipts; outcome pricing works	Institutions trust an independent verifier
Likely obstacles	Price pressure at the SMB end	Enterprise procurement; talent	Standards politics; cross-border law
Strategic consequence	Profitable, acquirable company	Category leader in India's regulated operations	Durable infrastructure company
Decisions needed early	Pack discipline; entity; pricing	Proof open vs closed; cofounder; funding path	Governance and neutrality design from Phase 2

External signposts that shift the odds

IF YOU SEE...	IT FAVOURS	ADJUST BY
A regulator issues audit or oversight rules specific to AI agents	B → C	Accelerate the Proof specification; hire regulatory lead
A global platform launches native agent execution for Indian hospitals or insurers	A	Become their executor partner; deepen cross-organisation moat
UPI agent protocol live with enterprise mandates at scale	B	Ship Mandate as a product
A major agent incident in Indian health or BFSI	B (trust demand) or A (slowdown)	Lead with the safety record; publish the incident-free history
Insurers start requiring machine-verifiable claim evidence	C	Offer Proof as the format; go all-in on payer network



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SECTION 27

Failure & Risk Tree

34 failure modes. Each row gives the trigger and failure, the early-warning signal, the consequence, and the mitigation with the contingency (↳).

#	CATEGORY	FAILURE / TRIGGER	EARLY WARNING	CONSEQUENCE	MITIGATION ↳ CONTINGENCY
01	Product	Horizontal sprawl: packs before references	More than 2 packs in build with fewer than 10 references	No reference anywhere; burn	Pack gate (Section 16) ↳ Freeze all but Health + one pack
02	Product	SMB tier undercut by Meta Business Agent	Falling trial-to-paid conversion	Revenue base erodes	Reposition on integrations and payments ↳ Sunset the lowest tier
03	Product	Pack needs custom code per client	Custom work above 30%	Services economics	Configuration-first design ↳ Stop sales; productise
04	Technology	Two codebases never converge	Duplicate features shipped twice	Half-speed engineering	Strangler plan (Section 13) ↳ Freeze one side
05	Technology	Scale limits (Supabase-only, schedulers)	Latency or lock contention at peaks	SLA breaches	Load tests before enterprise deals ↳ Dedicated instances
06	Technology	Browser automation breaks on portal changes	Rising failure rate per connector	Silent workflow failures	Verification read-back; monitoring ↳ Fallback to human queue
07	AI	Hallucinated extraction or summary	Accuracy drift in sampled reviews	Wrong data acted on	Deterministic parsers; confidence thresholds ↳ Disable AI step; human review
08	AI	Model deprecation or price shock	Provider notices; cost per task up	Margin loss; outages	Router with evaluated fallbacks ↳ Switch provider in a day
09	AI	Agent exceeds authority	Policy-violation alerts	Financial or legal harm	Mandate limits; T3 human gates ↳ Kill switch per agent
10	Security	Tenant data leak	Isolation test failures	Existential for trust	Row-level isolation tests in CI; pen tests ↳ Incident runbook; notify within DPDP timelines
11	Security	Prompt injection through documents or messages	Anomalous tool calls	Unauthorised actions	Tool allow-lists; content isolation ↳ Quarantine inputs
12	Compliance	DPDP non-compliance at May 2027	Gaps in consent or notice audit	Penalties; lost deals	Compliance sprint in Phase 0 ↳ External counsel review
13	Compliance	Clinical-boundary breach (NMC)	Firewall misses in testing	Legal liability	Firewall tests; clinical advisor ↳ Suspend feature
14	Enterprise sales	Endless unpaid pilots	Pilots over 90 days without price	Cash burn	Paid pilots with success criteria ↳ Walk away
15	Enterprise sales	Security review failure	Questionnaire gaps	Lost deals	ISO 27001 path; VAPT; DPA ↳ Partner-hosted option



Failure & risk tree (continued, 2/2)

#	CATEGORY	FAILURE / TRIGGER	EARLY WARNING	CONSEQUENCE	MITIGATION & CONTINGENCY
16	Enterprise sales	Champion leaves the account	Single contact	Renewal risk	Two contacts per account ↳ Executive sponsor outreach
17	Pricing	Anchored to about \$0.10 per action	Buyers compare with suite rate cards	Low revenue per workflow	Price outcomes and assurance ↳ Bundle into platform fee
18	Pricing	90-day payment terms starve cash	Receivables above 60 days	Runway crisis	Setup fees; annual prepay discounts ↳ Receivables financing
19	Competition	HMS/TPA software vendor bundles similar features	Feature announcements	Distribution lost	Partner early as their action layer ↳ White-label
20	Competition	Suite ships cross-tenant trust features	Launch news	Category squeezed	Open Proof spec; integrate with them ↳ Specialise in India DPI flows
21	Capital	Raise on story, judged on numbers	Missed milestones after the round	Down-round; control loss	Raise after gates only ↳ Cut to profitable core
22	Capital	Runway ends before enterprise revenue	Less than 6 months runway	Shutdown	Keep SMB revenue; lean team ↳ Bridge from angels or revenue financing
23	Talent	No commercial cofounder	Founder still doing all sales in 2028	Growth ceiling	Recruit by mid-2027 ↳ Fractional sales leader
24	Talent	Hiring from a tier-3 location	Offer rejections	Slow team build	Remote-first; Hyderabad hub ↳ Contract teams
25	Brand	Trademark opposition	Notices; confusion	Forced rename	Search and file now ↳ Distinct compound mark
26	Brand	Overclaims exposed	Customer or auditor challenge	Credibility loss (the Builder.ai lesson) [512]	Claim hygiene; measured data only ↳ Public correction
27	Scaling	Founder-led delivery doesn't scale	Founder in every onboarding	Throughput cap	Implementation playbooks ↳ Partner implementers
28	Data	No outcome data captured	Missing baselines	No ROI proof; no moat	Instrument before launch ↳ Retro-measure with clients
29	Integrations	Partner API or portal access revoked	Access errors; policy changes	Workflow outage	Multiple integration paths ↳ Manual fallback queue
30	Reliability	Missed SLA at a flagship client	Rising incident count	Reference lost	SLOs, on-call, status page ↳ Service credits; root-cause report
31	Customer trust	Automation goes too far (the Klarna lesson) [513]	Satisfaction drops; complaints	Churn	Always-a-human option ↳ Lower autonomy tier
32	Platform dependence	Meta policy, pricing or ban	Review delays; number restrictions	Channel outage	Purpose-bound flows; voice, SMS and web fallback ↳ Switch channel per tenant
33	Regulation	New AI rules restrict autonomous actions	Consultations and drafts	Rework	Tiered autonomy; Proof by design ↳ Lower default autonomy
34	Market timing	Buyers not ready for cross-organisation workflows	Payers stall after pilots	Network thesis delayed	Hospital-side value first ↳ Stay Scenario A longer



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SECTION 28

Build vs Buy vs Partner

CAPABILITY	DECISION	REASONING
Foundation models	INTEGRATE	Commodity; the router keeps independence
Speech, voice, Indic languages	PARTNER	Specialists lead; not Kriya's moat
OCR / document AI (scans)	INTEGRATE	Models improve fastest here
Deterministic parsers for known templates	BUILD	Accuracy and zero-retention guarantees
Payments and UPI agent protocols	INTEGRATE	Rails owned by NPCI and payment firms
Messaging channels	INTEGRATE	Meta and CPaaS; keep channel-portable
Human SSO and agent identity	INTEGRATE	IdPs (Entra Agent ID, Okta, Supabase auth) own identity [S19]
Delegated authority (Mandate)	BUILD	Core IP: domain-specific limits and consent
Policy engine and firewalls	BUILD	Core IP
Agent runtime / orchestration	BUILD thin	Omnitask exists; stay on open standards
Durable workflow execution	BUILD → evaluate	Own now; consider open-source engines at scale
Computer-use / browser execution	INTEGRATE + BUILD wrapper	Models commoditise; verification is Kriya's
Health and insurance connectors	BUILD	Defensible depth in home value chain
ERP/CRM connectors	PARTNER	SIs and connector platforms
Data infrastructure	BUY (managed)	Supabase/Postgres now; India-region enterprise cloud later
Observability (traces, cost)	INTEGRATE	Open tooling
Outcome analytics	BUILD	Moat: outcome data
Security tooling (SIEM, secrets, VAPT)	BUY / PARTNER	Specialists
Proof ledger	BUILD	Core IP; consider external anchoring later
Certification and audits	PARTNER	Independent auditors add credibility

RULE OF THUMB

Build what decides, authorises, proves, or encodes domain knowledge. **Integrate** what talks, clicks, stores, or transports. **Partner** where credibility or distribution matters more than code.



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SECTION 29 Team Evolution

FUNCTION	2026	2028	2030	2033	2036	2040
Total headcount HYPOTHESIS	1–3	8–15	40–100	150–300	400–800	Scenario A: 150–300 · C: 1,000+
Engineering & agent systems	Founder	4–6	15–35	50–100	150–250	Platform + verification network
AI research / evaluation	—	1 (evaluation)	2–4	8–15	20–40	Evaluation and safety science
Product & design	Founder	1	3–6	10–20	25–50	Per value chain
Security & trust	Contracted VAPT	1	3–5	10–20	30–60	Chief Trust Officer from 2031
Enterprise sales	Founder	1–2 (cofounder)	6–12	20–40	60–120	Country teams
Customer success / implementation	Founder	1–2	6–15	25–50	60–120	Partner-delivered majority
Domain experts (clinical, insurance)	Advisors	1 advisor + 1 hire	3–6	10–20	20–40	Councils per sector
Legal / compliance / regulatory	External counsel	Fractional	2–3	6–10	15–25	Standards and policy
Partnerships	—	Founder	2–3	6–10	15–30	DPI and assurance partners

Headcounts are planning hypotheses. They should be driven by revenue and gates, never ahead of them.

FIRST FIVE HIRES, IN ORDER

- **Commercial cofounder / enterprise lead**, ideally from hospital revenue cycle, insurance or health SaaS sales
- **Senior backend engineer** to own convergence and reliability
- **Implementation lead** to take onboarding off the founder
- **Security and compliance lead** (can start fractional)
- **Clinical or insurance domain advisor**, then a hire

DON'T HIRE EARLY

- AI researchers before an evaluation need exists
- Large sales teams before a repeatable playbook
- Country managers before a partner-led pilot abroad
- Marketing before case studies

WHAT KIND OF COMPANY KRIYA IS AT EACH STAGE

2026: a founder-led product lab. **2028:** an enterprise software startup with a repeatable sale. **2030:** a network business with a partner ecosystem. **2033+:** an infrastructure and assurance company where trust operations matter as much as engineering.



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SECTION 30

Capital Strategy

STAGE	SOURCE	TRIGGER	USE OF FUNDS	WATCH-OUT
Now → mid-2027	Bootstrapped; cloud and model credits; incubator or government seed programmes	Always	Prove Phase 0	Slow if founder time is scarce
Optional pre-seed	Angels with healthcare or insurance networks	After 5+ paying customers and a pilot	Part-time help; certifications	Dilution for little money
Seed HYPOTHESIS	~\$1–3M from sector-aware funds	Phase 0 gates met	Team of 8–15; ISO 27001; packs	Expectations of rapid ARR
Series A HYPOTHESIS	Institutional	₹2–5 Cr ARR; NRR >110%; counterparty accepting Proof	Network build; enterprise sales	Must show repeatability
Strategic investor	Insurer, hospital group or payments company	Phase 2	Distribution and credibility	Neutrality risk for Phase 4 verification
Working capital	Revenue-based or receivables financing	When 60–90 day terms bite	Cash smoothing	Cost of capital

Engineering & AI infrastructure

Inference cost is pass-through (BYO key) or priced managed. Main costs are people and India-region hosting for enterprise tenants.

Enterprise sales

The most expensive line from Phase 1. Budget for long cycles and paid pilots that may not convert.

Security, legal, compliance

ISO 27001, VAPT, counsel and insurance. Lumpy costs; plan them before the first enterprise contract.

International expansion

Partner-led first. Don't fund a foreign entity before a partner pilot converts.

NON-NEGOTIABLES

1. Raise only after gates, not on narrative.
2. Report recognised revenue only; no “run-rate” inflation. Audited numbers from Phase 2.
3. Keep 12+ months of runway after every raise.
4. Choose investors who understand regulated sales cycles.



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SECTION 31

Kriya Autonomous Operations KPI Framework

DIMENSION	METRIC	DEFINITION	WHY IT MATTERS
Business	ARR (recognised)	Annualised contracted recurring revenue, live customers only	Growth truth
Business	Net revenue retention	Revenue from a cohort now vs 12 months ago	Expansion proof
Business	Gross margin	Revenue minus inference, hosting, messaging and delivery labour	Services-trap detector
Business	Sales cycle and win rate	Days from first meeting to signature; wins/opportunities	Go-to-market health
Product	Workflow automation rate	Share of cases completed without human touch	Autonomy
Product	Human escalation rate	Escalations per 100 cases, by reason	Safety and product gaps
Product	Resolution rate	Cases resolved to the verified end state	Real completion
Agent	Verified-action rate	Actions confirmed by target-system read-back	Core of the thesis
Agent	Tool-call reliability	Successful tool calls / attempts	Integration quality
Agent	Recovery rate	Failures recovered without human intervention	Resilience
Agent	Hallucination / extraction error rate	Errors per 1,000 extractions in sampled audits	Trust
Agent	Policy violations	Blocked or attempted out-of-policy actions	Governance
Operational	Hours automated; ₹ recovered or saved	Measured against customer baseline	ROI proof
Operational	Turnaround and response time	Per workflow, before vs after	Customer value
Technical	Latency; uptime; cost per task	p95 latency; monthly availability; ₹ per completed case	Reliability and margin
Technical	Model fallback rate; integration uptime	Share of calls rerouted; connector availability	Dependence risk

TARGETS POLICY

No benchmark targets are invented here. For each metric: **measure a 90-day baseline per customer**, then set targets with the customer. Two internal policy targets are stated as hypotheses: gross margin above 65–70% by the end of Phase 1, and zero tolerance for tenant-isolation or clinical-boundary incidents. **HYPOTHESIS**



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SECTION 32

36-Month Execution Plan

OCT – DEC 2026

0–90 days

PRODUCTS	Proof v0; tier repositioning; Taiyo pilot live
CUSTOMERS	≥5 paying tenants; 1 corporate batch
EXPERIMENTS	20 buyer interviews (HR, revenue cycle, TPA)
ARCHITECTURE	Health ↔ Omnitask contract; LLM fallback
HIRING	Part-time implementation help
PARTNERSHIPS	Taiyo; 1 HMS vendor conversation
REVENUE OBJECTIVE	Cover running costs
MILESTONES	Brand/entity decision; trademark search; claim hygiene done
KILL CRITERIA	No paying pilot by December → re-examine corporate offer

JAN – MAR 2027

3–6 months

PRODUCTS	Workforce Health v0 on Omnitask
CUSTOMERS	8–12 tenants; 1 enterprise LOI
EXPERIMENTS	Paid employer pilot via lab; claims shadow study at one hospital
ARCHITECTURE	Omnitask control plane for new flows
HIRING	Cofounder search active
PARTNERSHIPS	First broker conversation
REVENUE OBJECTIVE	Phase 0 gates
MILESTONES	Two case studies; DPDP readiness
KILL CRITERIA	<5 paying → fix ICP before new builds

APR – SEP 2027

6–12 months

PRODUCTS	Claims pilot; Comply add-on; Proof v1
CUSTOMERS	2–3 enterprise pilots paid
EXPERIMENTS	Outcome-based price test
ARCHITECTURE	Shared tenant, Proof and Mandate
HIRING	Cofounder or lead; senior engineer
PARTNERSHIPS	TPA willing to accept receipts
REVENUE OBJECTIVE	First ₹10 L+ contract
MILESTONES	Graduation → full-time; seed readiness
KILL CRITERIA	Pilots stall >90 days unpaid → change offer

OCT 2027 – MAR 2028

12–18 months

PRODUCTS	Claims v1 across one payer; Lens for any lab template
CUSTOMERS	3+ enterprise logos
EXPERIMENTS	Broker channel scale test
ARCHITECTURE	MCP server; load tests
HIRING	Security lead; implementation lead
PARTNERSHIPS	HMS vendor alliance
REVENUE OBJECTIVE	₹1 Cr ARR run-rate KRIYA SCENARIO
MILESTONES	Seed closed (if gates met); ISO 27001 underway
KILL CRITERIA	Custom work >30% → productise



APR – SEP 2028

18–24 months

PRODUCTS	Mandate v1; Outcome dashboards
CUSTOMERS	First hospital group network (multi-site)
EXPERIMENTS	Per-verified-action pricing
ARCHITECTURE	India-region dedicated deployments
HIRING	Sales team of 2–3; domain hire
PARTNERSHIPS	Second TPA; payment partner
REVENUE OBJECTIVE	₹2 Cr ARR KRIYA SCENARIO
MILESTONES	ISO 27001 certified; NRR measured
KILL CRITERIA	Enterprise win rate low → mid-market pivot

OCT 2028 – SEP 2029

24–36 months

PRODUCTS	Proof open spec draft; second pack decision (Pharma or Claims+)
CUSTOMERS	5+ enterprise logos
EXPERIMENTS	Gulf partner discovery
ARCHITECTURE	A2A gateway beta
HIRING	12–15 total
PARTNERSHIPS	Insurer or employer council
REVENUE OBJECTIVE	₹3–5 Cr ARR KRIYA SCENARIO
MILESTONES	Phase 1 gates → Series A decision
KILL CRITERIA	No counterparty accepts Proof → stay Scenario A

36-month scoreboard

MILESTONE	TARGET DATE KRIYA SCENARIO	UNLOCKS
Proof v0 on live traffic	Dec 2026	Category story with real evidence
Paid employer batch through Taiyo	Dec 2026	Workforce Health pack
Phase 0 gates met	Jun 2027	Full-time founder; seed conversations
Commercial cofounder or lead on board	Sep 2027	Enterprise pipeline
First counterparty accepting Proof (TPA/insurer)	Sep 2028	Network thesis
ISO 27001 certified	Sep 2028	Enterprise procurement
₹3–5 Cr ARR, NRR >110%	Sep 2029	Series A decision

WORKING BACKWARDS FROM 2040

Every item above builds one of the four things the 2040 end state requires: **outcome data, accepted proof, multi-party integrations** and **trusted relationships with regulated buyers**. If a proposed task builds none of them, it waits.

FOUNDER TIME BUDGET (OCT 2026 – JUN 2027)

Protect roughly: 40% selling and customer work, 35% product and convergence, 15% degree, 10% hiring and partnerships. Anything outside Kriya comes out of the 35%, which is why the stop-doing list matters. **HYPOTHESIS**



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SECTION 33

The 2040 End State

A plausible 2040, not science fiction. Everything below is **KRIYA SCENARIO**.

What does Kriya sell?

Verified execution of regulated business actions across organisations, sold as outcomes (per settled claim, per compliant case) plus verification and certification of other parties' agents.

Who buys it?

Insurers, hospital networks, large employers, pharma, lenders and their counterparties. Regulators and auditors use it to inspect; they don't buy it.

What does the platform actually do?

It receives intents from people, systems and agents; checks authority and policy; executes; verifies; issues receipts; and escalates exceptions to accountable humans.

An employee's experience

A claims officer manages a queue of exceptions only: cases where policy, evidence or the counterparty disagree. Routine work closes itself with receipts attached.

An enterprise's experience

The COO sees outcomes, cost per case, autonomy levels and incidents per workflow. The CISO sees every agent's authority and every action's proof in one place.

Agent meets another company's agent

A hospital's agent submits a claim to an insurer's agent over A2A. Kriya checks both mandates, verifies the evidence and records a shared receipt. Settlement triggers on payment rails with no human re-keying.

What role does WhatsApp play?

One of several human-facing surfaces for citizens: important in India and the Global South, but no longer the product. Most traffic by volume is agent-to-agent.

Websites, voice, APIs

Voice for inclusion; web for oversight consoles; APIs and agent protocols for the majority of actions.

What Kriya owns

The authority model, policy engines, proof format, verification network, domain ontologies and outcome data.

What Kriya integrates

Models, channels, identity providers, payment rails, systems of record, computer-use execution.

Why it is hard to replace

Counterparties accept its receipts. Contracts and audits reference them. Years of outcome data calibrate its policies. Liability partners underwrite actions it verifies. Replacing Kriya would mean re-establishing trust between many institutions at once. That is a network position, not a feature.



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SECTION 34

The 10 Most Important Decisions

DECISION	WHY IT MATTERS	OPTIONS (+ ADVANTAGE · - RISK)	EVIDENCE TO GATHER	DEADLINE
1 · Legal entity and master brand	Contracts, trademarks and investor structure attach to it	A: Xylarc AI stays the entity, Kriya is the brand (+ fast; - two names) · B: rename company to Kriya (+ clarity; - name crowding, cost)	Trademark search result; accountant advice	Dec 2026
2 · Codebase convergence path	Determines engineering speed for 2 years	A: strangler onto Omnitask (+ safe) · B: rewrite (- months of frozen sales) · C: keep separate (- duplicate work)	Contract layer effort estimate	Jan 2027
3 · First enterprise wedge	Sets team, partners and data	Workforce Health (+ built, fast) · Claims/revenue cycle (+ bigger budget, - longer) · Comply (+ deadline, - crowded)	20 interviews; one paid pilot	Mar 2027
4 · Fate of the SMB tiers	Meta commoditises basic booking	Keep / reposition / sunset lowest tier	Conversion and churn trend	Mar 2027
5 · Commercial cofounder	Largest single lever on odds	Cofounder (+ commitment, - equity) · senior hire (+ speed, - cost) · fractional (+ cheap, - depth)	Candidate pipeline	Jun 2027
6 · Channel strategy	Platform dependence	WhatsApp-first with fallbacks vs full omnichannel from start	Meta review outcome; customer demand	Jun 2027
7 · Model and hosting strategy	Margin, residency, independence	BYO key · managed inference · sovereign option	Enterprise security requirements	Jun 2027
8 · Funding path and timing	Control and pace	Bootstrap longer vs seed after gates vs strategic investor	Phase 0 gate results	Sep 2027
9 · Proof: open specification or closed	Category creation vs short-term protection	Open spec (+ adoption, - copying) · closed (+ control, - slower adoption)	Counterparty willingness; advisor input	Dec 2028
10 · First international market	Capital and focus	Gulf via partners · Southeast Asia · none before Phase 3	Partner pilot economics	2029–2030

No option is pre-selected where the evidence isn't in yet. My leanings are in Section 35 and marked as hypotheses.



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SECTION 35

CEO Final Synthesis

The final CEO test, and the revisions it forced

CHALLENGE	HONEST ANSWER	REVISION MADE
If I were investing my own money, what would make me reject this?	A solo, part-time founder with two codebases and no measured customer outcomes.	Phase 0 gates focus entirely on proof and convergence.
Most dangerous assumption?	That counterparties (TPAs, insurers) will accept and value third-party receipts.	Test it in months 6–12 with one TPA before building the network.
What will foundation-model companies commoditise?	Understanding, extraction, conversation, computer use.	Browser automation downgraded as a moat; the router kept thin.
What will competitors copy within 12 months?	Booking flows, dashboards, orchestration, multilingual chat.	Differentiation moved to Proof, Mandate, outcome data.
What can Kriya own?	Authority policy, proof format, domain ontologies, cross-organisation integrations in health.	These are the only “BUILD” items.
What can’t Kriya realistically own?	Models, channels, identity, payment rails, systems of record.	All integrate or partner.
Which idea sounds futuristic but has no buyer?	“AI employee OS” or “autonomous enterprise workforce” as a category.	Rejected as a category; kept as a later narrative.
Which boring problem could become massive?	Claim deductions and query loops; benefits administration; consent handling.	These became the first three packs.
Where is the hidden enterprise budget?	Headcount and outsourcing in revenue-cycle, TPA, HR-benefits and legal operations.	Pricing anchored to savings, not software lines.
Strongest India-specific advantage?	Health digital rails (ABDM/NHCX), UPI agent payments, DPDP, WhatsApp-native citizens.	India-first packs built on DPI connectors.
Where could Kriya become globally relevant?	Gulf and Southeast Asia health and insurance networks; later, agent verification.	Partner-led international timing.
What would destroy Kriya?	Sprawl, a trust incident, or an overclaim exposed.	Pack gate, risk tiers, claim hygiene.
What would make Kriya very hard to replace?	Being the receipt that many institutions accept.	Proof open specification plus network strategy.



CEO-level conclusion

	CONCLUSION
A · Current strategic position	A technically strong, commercially unproven healthcare action system (Kriya Health) plus an advanced, customer-less agent runtime (Kriya Omnitask), squeezed by Meta's native agent below and suites above.
B · Most important transformation	From selling "agentic chatbots for hospitals" to owning verified action : authorised, verified, provable execution across organisations.
C · Architecture to build	Omnitask as runtime and policy engine; Proof, Mandate and Lens as owned trust modules; Reach and the router as thin integration; strangler convergence of the two codebases.
D · Vertical expansion logic	Follow counterparties and document flows: providers → employers → claims and revenue cycle → consent → pharma → other claims-heavy sectors. New packs only with 3 paying design partners.
E · Enterprise strategy	Dated, budgeted, multi-party problems; paid pilots with success criteria; channels through labs, brokers, HMS vendors and TPAs; pricing against headcount and leakage.
F · 2026–2030 priorities	Measured references; convergence; Workforce Health and Claims; ISO 27001 and DPDP; commercial cofounder; seed after gates; first counterparty accepting Proof.
G · 2030–2040 direction	Cross-organisation network → autonomous regulated operations → verification of third-party agents → global verified-action infrastructure, with partners abroad.
H · Principal risks	Founder bandwidth and sprawl; counterparties rejecting receipts; incumbent bundling; liability incidents; platform dependence on Meta; capital timing.
I · Assumptions to validate	Buyers pay for proof, not just savings; TPAs and insurers accept receipts; hospital deductions are large enough to fund a price; Omnitask performs at enterprise load; Gulf partners exist.
J · Next actions	1) Record the entity and brand decision and start a trademark search. 2) Ship Proof v0 on live bookings and refunds. 3) Take the Taiyo pilot live with a paid employer batch. 4) Run 20 buyer interviews. 5) Build the Health ↔ Omnitask contract layer. 6) Clean every public claim. 7) Start the cofounder search.

LEAN, STATED AS HYPOTHESES

On the Section 34 decisions, my leanings are: keep Xylarc AI as the legal entity for now and Kriya as the only public brand; strangler convergence onto Omnitask; Workforce Health as the fastest first wedge with Claims as the bigger second; reposition the SMB tiers now and decide on sunseting in March 2027; and open the Proof specification once one counterparty accepts receipts.



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SECTION 36

Sources & Research Notes

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- S11 Profiles of companies named Kriya/Criya: Wikipedia, Preqin, CB Insights, Inc42, YC**
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- S17 Computer-use agents: UiPath Business Wire release (Jan 2026, OSWorld-Verified #1); practitioner review (managed-code.com, 2026); OSWorld 2.0 (arXiv, Jun 2026)**
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- S21 ABDM: PIB factsheet (Jul 2026); Mission Steering Group coverage, GKToday and Indian Pharma Post (Jul 2026)**
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- S22 WhatsApp market structure: Rest of World (2024); Mobile Ecosystem Forum (2025)**
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- S24 MeitY India AI Governance Guidelines (5 Nov 2025): AZB & Partners, S.S. Rana analyses**
India · policy

Research notes

FINDING THAT CHANGED THE STRATEGY: CLAIMS

Official data shows high insurer compliance with IRDAI timelines: 96.69% of final authorisations within 3 hours [S23]. The claims opportunity is therefore **not** insurer SLA penalties, as the earlier blueprint suggested. It is hospital-side cost: document assembly, query cycles, deductions and reconciliation, plus TPA processing cost. Plays and pricing were revised accordingly.

FINDING THAT CHANGED THE STRATEGY: THE SMB TIER

Meta Business Agent (global June 2026, India July 2026) does appointment booking and lead qualification natively [S14]. Kriya Health's value must move to what Meta doesn't do: integrations, payments with holds and refunds, report delivery, the clinical firewall, multi-branch operations and proof.

FINDING THAT CHANGED THE STRATEGY: BROWSER AUTOMATION

Computer-use agents are improving fast, with top OSWorld-Verified results in 2026 [S17]. Kriya Reach is therefore a thin integration layer with a verification wrapper, not a moat.

EVIDENCE INSUFFICIENT

1. Size of hospital revenue-cycle and claim-deduction losses in India: no reliable public figure found.
2. Employer spend on benefits administration in India.
3. Willingness of TPAs and insurers to accept third-party receipts.
4. Kriya's own outcome metrics: none measured yet.
5. Gulf market specifics for health-insurance workflows: to be verified with partners.

Figures are as reported by the listed sources on 30 September 2026. Verify originals before quoting them to investors or customers. This document is strategy, not legal, financial or investment advice.



KRIYA · VERIFIED ACTION

AI that acts. Proof that it acted right.

Kriya Health proves it. Kriya Omnitask runs it. Kriya Proof makes others trust it.

NEXT 90 DAYS

Proof v0 · Taiyo pilot · 20 interviews

BY 2029

Verified health enterprise

BY 2040

Global verified-action infrastructure